

# Safeguarding Medical Examination Pack

Name:

S Number:

D.O.B:

NHS no:

Address:

**Patient Information:**

First name:

Surname:

D.O.B

Age:

S Number:

Gender:

Ethnicity:

Childs first language:

Is child on a child protection plan?

Does child have a social worker, if so their name and telephone?

Who has parental responsibility?

Telephone numbers for those with parental responsibility

**Reason for CP medical:**

Reason CP medical commenced (If examination requested, who by and why)

Referrer:

Position:

Contact no:

Plans for undertaking a child protection medical should be discussed with social care. Please document initial discussion with social care. If discussion did not take place, please document why

Name of social worker :

Telephone no:

Patient information, reason for medical

## Patient sticker if available

Name:

S Number:

D.O.B:

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Address:

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## Examination details:

Place of examination:

Date of examination:

Time:

Examining Dr and grade:

Lead Paediatric Consultant:

## Persons present:

(including relationship to child / agency)

A Professional chaperone must be used for a child protection medical. This can be a junior doctor, nurse or health care assistant.

Name of chaperone:

Chaperones job title:

## Interpreter

Is an interpreter required:

Yes

No

If an interpreter is required it is important that they are a professional interpreter please document:

Patients first language:

Language of interpreter:

Name of interpreter:

Service used:

Section One - Examination details and persons present

**Patient sticker if available**

**Name:**

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**Consent for medical assessment:**

If available online consent should be used. Consent must be taken from someone with parental responsibility

I understand that there are concerns about my child's safety or well-being (child protection concerns) and that a medical examination by a children's doctor is required as part of an assessment into these concerns. I understand that this examination is to ensure that the child receives any medical treatment they may require for pain or injury but also to document carefully any marks / bruises / injuries / findings of concern in order for the doctor to consider how they may have been caused. It has been explained that this may include some or all the following (delete if not applicable).

a) A full medical history and complete examination;

b) Taking of notes and photographs/digital images for recording and evidential purposes (including second opinions from medical experts and peer review)\*;

c) Investigations if required which may include x rays, blood tests, scans, or others\*

d) I understand and agree that the doctor/nurse may provide a statement/report for the police, social services, paediatric services and the GP;

e) I understand and agree that a copy of the medical notes may be given to professionals involved in the case (e.g. police / lawyers) and may be used in a court;

f) (Optional) I agree to the use of anonymised photographs/videos/digital images/medical notes for teaching, audit and research;

g) I have been advised that I may withdraw my consent at any time.

\*If electronic consent is not used, further written consent will be required for these investigations as per trust guidelines

**Name:**

**Relationship to child:**

**Date**

*Signature*

**Young Person's name:**

**Date:**

*Signature*

**Doctors name:**

**Position:**

**Date:**

*Signature*

Patient sticker if available

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History of events from professionals requesting medical

Clearly document **who** is giving the history:

Section Two - History from Professionals involved in case

Patient sticker if available

Name:

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History of events from parents / carers and child if able

Clearly document **who** is giving the history, **when** and **where** events occurred, whether any **witnesses** and **what happened afterwards**. Child should be spoken to without carer present, if not possible document why.

Section Two - History from Parent / Carer / Child

**Patient sticker if available**

**Name:**

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**D.O.B:**

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**Address:**

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**Past medical history**

Birth

Gestation:

Delivery:

Birth

Weight:

Resuscitation / NNU

**Previous injuries / illnesses:** document birth marks and review red book if available

Drug history

Allergies

Immunisation  
status

**Section Two - Medical history**

Name:

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## Social / family history

### Family tree:

- Names and ages of parents
- Who lives with child / has regular contact (siblings names, ages) - Genogram
- Have children previously been on **CP plan**
- **School / nursery attendance**

### Family history (clotting disorders)

### Mother

Name:

Age:

Occupation:



### Father

Name:

Age:

Occupation:

Any other significant adults?

Residence / contact issues?

### Siblings

|                  | 1 | 2 | 3 | 4 | 5 |
|------------------|---|---|---|---|---|
| Name             |   |   |   |   |   |
| Sex              |   |   |   |   |   |
| Age / D.O.B      |   |   |   |   |   |
| Nursery / school |   |   |   |   |   |
| Health           |   |   |   |   |   |

## Social / developmental history

Name:

S Number:

D.O.B:

NHS no:

Address:

### Social

Any smokers in family?

Any alcohol use?

Any drug use?

Housing / housing problems?

Domestic violence?

Does family have social worker / are they known to social services

### Development

#### Gross motor

Rolls over 8 – 18 weeks  
Sits alone 18 - 30 weeks  
pulls to stand 6 - 10 months  
walks holding on 7 - 13 months  
walks alone 11 - 14 months  
jumps feet together 12 - 20 months  
kicks ball 14 - 24 months  
pedals tricycle 18 - 38 months

#### Fine motor

follows person with eyes 6 - 12 weeks  
holds hands together 8 - 16 weeks  
reaches out for rattle 10 - 20 weeks  
rattle from hand to hand 22 - 40 weeks  
palmar grasp 24 - 32 weeks  
pincer grip 9 – 12 months  
circular scribble 12 - 24 months  
copies circle 24 - 42 months  
copies cross 32 - 58 months  
can draw a person 32 - 58 months

#### Speech, language and hearing

turns to sound at ear level 1 - 3 months  
squeals 6 - 20 weeks  
turns to voice 14 - 30 weeks  
'dada', 'mama' to anyone 6 - 10 months  
'dada', 'mama' to parents 40 - 60 weeks  
3 words + 'dada'/'mama' 12 - 20 months  
puts two words together 14 - 24 months  
gives first and last name 2 - 4 years

#### Social and educational

Smiles spontaneously 1.5 – 3 months  
Finger feeds 4 – 8 months  
Shy with strangers 5 – 10 months  
Dry during day 18 – 38 months  
No separation anxiety 20 – 50 months  
Knife and fork 32 – 50 months



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## General examination

Growth

Weight:

Centile

Height:

centile

Head

circ:

Centile

General examination:

Dress, clothing, hygiene:

Demeanour :

Interaction (with both parents and staff):

Observations:

Temp

HR

RR

Sats

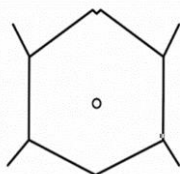
BP

Respiratory:



Cardiovascular:

Abdominal:



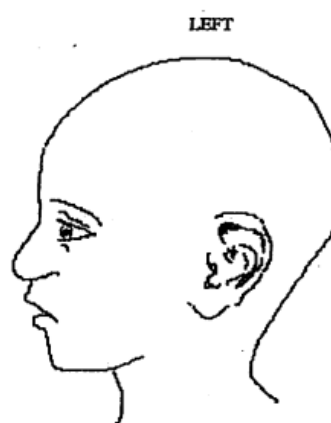
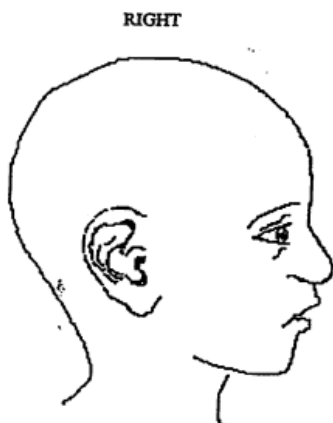
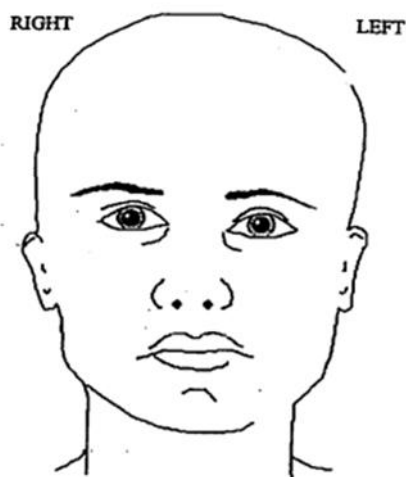
Neurological

## Body Map - Face

When completing body map

Number each injury and note:

- Size (measured), Shape, Colour of lesions,
- Indicate if photograph on nerve centre



|                         |     |
|-------------------------|-----|
|                         | Yes |
| No abnormality detected |     |

Dr Name:

Chaperone:

Signature:

Signature:

Date:

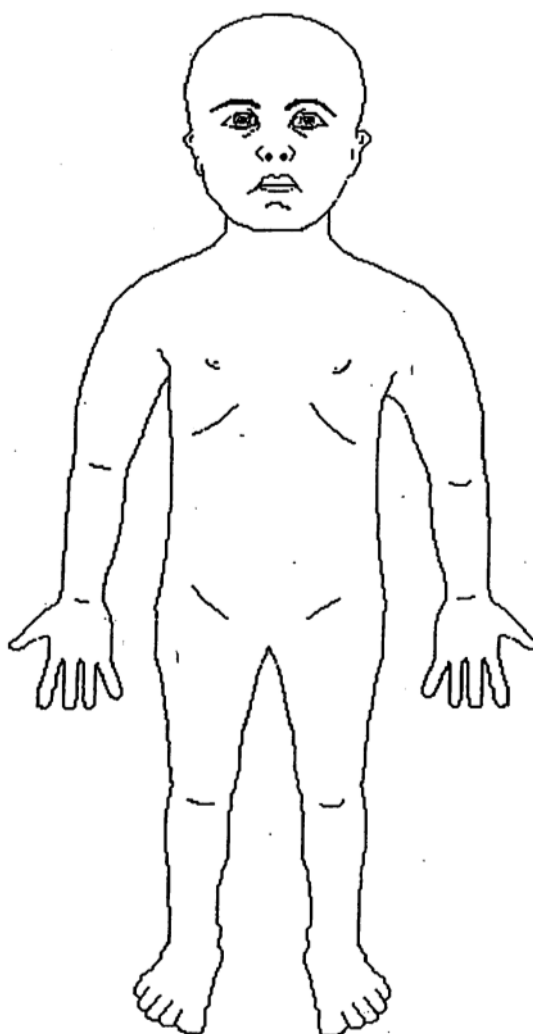
Time:

# Body Map – Torso and limbs front

When completing body map

Number each injury and note:

- Size (measured), Shape, Colour of lesions
- Indicate if photograph on nerve centre



No abnormality detected

Yes

Dr Name:

Signature:

Date:

Chaperone:

Signature:

Time:

Name:

S Number:

D.O.B:

NHS no:

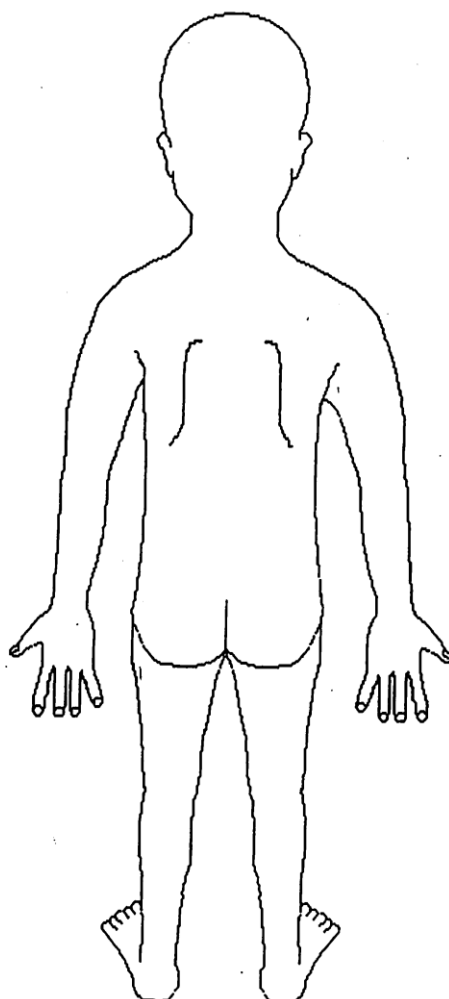
Address:

## Body Map – Torso and limbs back

When completing body map

Number each injury and note:

- Size (measured), Shape, Colour of lesions
- Indicate if photograph on nerve centre



No abnormality detected

Yes

Dr Name:

Chaperone:

Signature:

Signature:

Date:

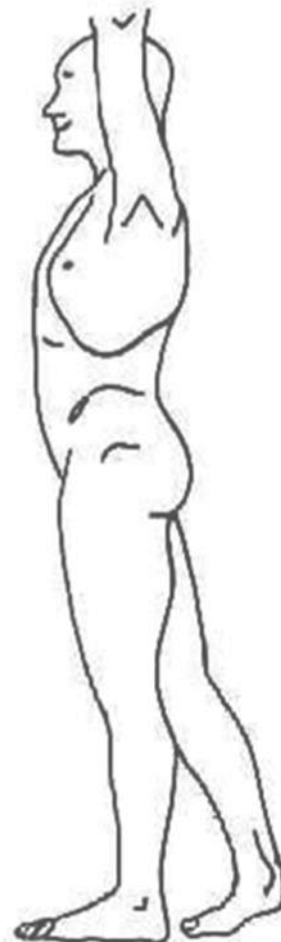
Time:

## Body Map – Torso and limbs sides

When completing body map

Number each injury and note:

- Size (measured), Shape, Colour of lesions
- Indicate if photograph on nerve centre



Yes

No abnormality detected

Dr Name:

Chaperone:

Signature:

Signature:

Date:

Time:

## Body Map – Hands and feet

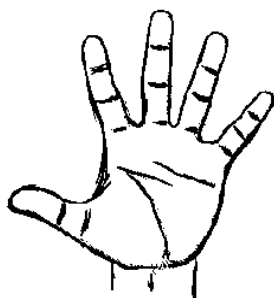
When completing body map

Number each injury and note:

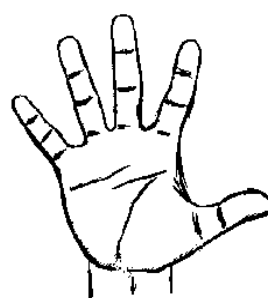
- Size (measured), Shape, Colour of lesions
- Indicate if photograph on nerve centre



LEFT HAND



RIGHT HAND



LEFT FOOT



RIGHT FOOT



|                         |     |
|-------------------------|-----|
|                         | Yes |
| No abnormality detected |     |

Dr Name:

Chaperone:

Signature:

Signature:

Date:

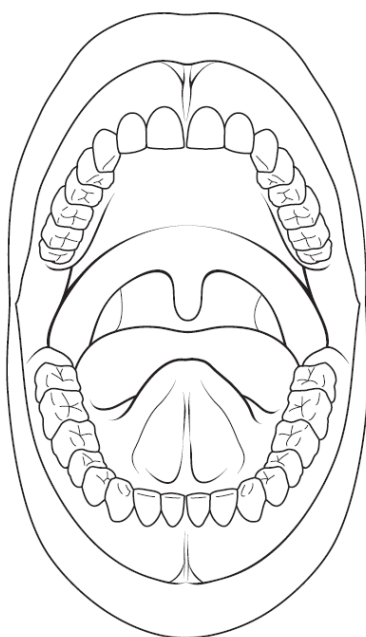
Time:

## Body Map – Mouth

When completing body map

Number each injury and note:

- Size (measured), Shape, Colour of lesions
- Indicate if photograph on nerve centre



No abnormality detected

Yes

Dr Name:

Chaperone:

Signature:

Signature:

Date:

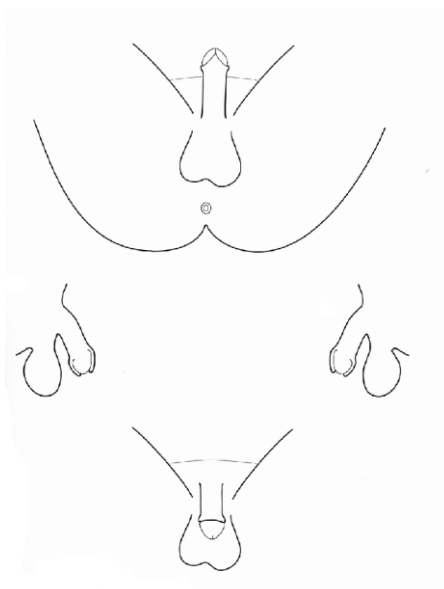
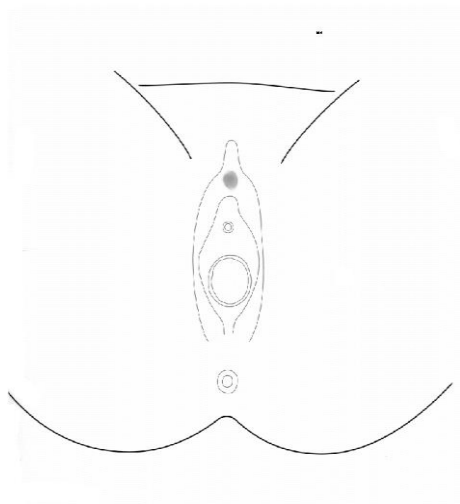
Time:

## Body Map – Genitals

When completing body map

Number each injury and note:

- Size (measured), Shape, Colour of lesions
- Indicate if photograph on nerve centre



No abnormality detected

Yes

Dr Name:

Chaperone:

Signature:

Signature:

Date:

Time:



Name:

S Number:

D.O.B:

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Address:

## Injuries found on examination and explanations offered

Document explanations offered and whether you feel explanations match clinical findings. Explanations should always be considered in context of child's development. Document any discussions with consultant.

| Injury<br>( number and site of injury) | Explanation from carer and child if able | Interpretation |
|----------------------------------------|------------------------------------------|----------------|
|                                        |                                          |                |
|                                        |                                          |                |
|                                        |                                          |                |
|                                        |                                          |                |
|                                        |                                          |                |
|                                        |                                          |                |
|                                        |                                          |                |
|                                        |                                          |                |
|                                        |                                          |                |

**Summary and opinion**

This is a summary and opinion based upon clinical findings and investigations to date. This opinion may change when further investigation results are available, and discussions have taken place with both social care and the police.

Name:

S Number:

D.O.B:

NHS no:

Address:

**Summary of medical history and clinical findings****Opinion and level of concern****Safeguarding referral**

Yes

A safeguarding referral must be made on ICE for all safeguarding medicals. This should be done even if following review there are no safeguarding concerns. Please tick to show referral made

In office hours please phone 15770 and speak to one of the safeguarding team. Out of hours please leave a message

Name

Signed

Date

Time

**Patient sticker if available**

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**Name:**

**S Number:**

**D.O.B:**

**NHS no:**

**Address:**

**Date:**

**Time:**

**Name of social worker:**

**Contact number:**

**Discussion with social care**

Has this case been discussed with Social services?  
(All Red cases should be discussed with social services prior to discharge. If you have any concerns about the child following the medical contact social services and inform them)

Leicester City: 0116 454 1004

Leicester county: 0116 305 0005

Section Five - Discussion with Social care

Name

Signed

Date

Time

**Investigations**

Use the guide below for organising investigations

Name:

S Number:

D.O.B:

NHS no:

Address:

**Bloods**

| Test:             | Taken: | Results: |
|-------------------|--------|----------|
| FBC               |        |          |
| U+E               |        |          |
| LFT               |        |          |
| Bone Profile      |        |          |
| Vit D +PTH        |        |          |
| Coag              |        |          |
| Extended clotting |        |          |

**Radiology**

| Age     |             | Investigation                | Requested | Results |
|---------|-------------|------------------------------|-----------|---------|
| <1 year |             | CT Head >1 only if indicated |           |         |
|         | 1 – 2 years | Skeletal survey              |           |         |
|         | > 2 years   | Focussed X rays              |           |         |
|         |             |                              |           |         |
|         |             |                              |           |         |
|         |             |                              |           |         |

Name

Signed

Date

Time

**Patient sticker if available**

**Name:**

**S Number:**

**D.O.B:**

**NHS no:**

**Address:**

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## Ophthalmology

Ophthalmology reviews should take place on all children less than 2 as part of medical assessment.

Section Six - Ophthalmology

Name

Signed

Date

Time

Name:

S Number:

D.O.B:

NHS no:

Address:

## Report

A report must be dictated on DIT3 by searching for patient and selecting safeguarding when dictating letter. Phone 5770 and leave message to inform that report done. Below is a basic outline to help you dictate.

**Re: Name, DOB, Address**

**Place of examination:**

**Date and time of Examination:**

**Examining Doctor:**

**Persons present:** Chaperone name and position.

**Consent:** Who consent obtained from.

**Reason for examination and if examination requested who by:**

**History:** Provide history for any injury and who has provided history.

**Medical history:** past medical history of bruising or fractures?

**Social and family history:** Occupations of parents, any drug or alcohol use. Any history of domestic violence. Whether anyone in family has social worker and their name.

**Developmental history:** Full developmental history

**Examination:** Cleanliness, dress, Weight and height. Systems examination. Clearly document any injuries (position, size, and colour). Comment on interaction with parents.

**Investigations:** Bloods: Skeletal survey: CT Head: Ophthalmology examination:

**Summary:** Summary of injuries and explanations offered.

**Opinion:** Opinion whether injuries in keeping with explanations offered and likelihood of these being non accidental.

The above medical opinion has been given in the context of the information known at the time as reflected in the report. It is important to consider the above medical opinion in the context of information known by other agencies. If new information comes to light as part of the multiagency assessment that may impact on the medical opinion significantly, please inform me so that I can send an addendum to the report.

## Checklist

Please complete checklist to ensure that all child protection medical tasks have been completed.

Name:

S Number:

D.O.B:

NHS no:

Address:

Social care has been contacted and advised of plan to perform child protection medical ☐

Consent has been obtained for medical. ☐

Child protection medical history and examination have been completed. ☐

Safeguarding ICE referral has been made. ☐

Safeguarding team has been spoken to / message left. ☐

If not already aware, social services have been informed that medical examination has been completed. ☐

Bloods have been taken including PTH and extended clotting if needed. ☐

Consent has been obtained for skeletal survey / CT head under sedation if required. ☐

ICE referrals have been made for radiological investigations. ☐

Ophthalmology review has been requested. ☐

Results of bloods, radiology and Ophthalmology reviews chased. ☐

Report dictated and opinion made. ☐

Attendance at strategy meeting. ☐

Clear plan for safe discharge made. ☐

## USEFUL CONTACT NUMBERS

### UHL Safeguarding Children Team:

- Office Telephone: 0116 258 5770 (15770)
- UHL Email: [child.protectionteam@uhl-tr.nhs.uk](mailto:child.protectionteam@uhl-tr.nhs.uk)

### Social care:

Leicester City Social Care: 0116 454 1004 (24 hour service)

Leicestershire County Social Care: 0116 305 0005 (24 hour service)

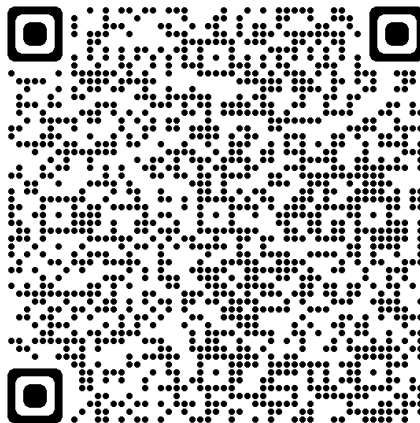
Rutland Social Care: 01572 758 407 (office hours only) (contact Leicestershire County outside office hours)

Police Child Abuse Investigation Unit: 0116 428 5500

### Information for parents or carers

Please scan this QR code to access the patient information leaflet

**‘What to expect when we feel a child protection medical examination is needed’**



Alternatively the leaflet can be downloaded and printed from the YourHealth library